pushed by the HMO, so as to avoid the costs associated with illness. The choice of treatments will be done with a view to keeping you healthy.

At present, the conversation between a doctor and a patient in India is a transactional one, where symptoms are described and treatments are explained. The time in that room is a powerful opportunity to change behaviour. A few minutes spent by the doctor evangelizing better behaviour tends to have a significant impact on behaviour and health. As an example, a doctor might say: 'I'm prescribing a programme of exercise for you, and I want you to come back to me in three months and we will look at the improvements in your cholesterol numbers.' This would be quite motivating for most patients. Such practices that result in improved health are incentive-compatible for the HMO, as they yield reduced costs and higher profit.

A doctor in an HMO who sees a surge of an infectious disease in her neighbourhood would have the incentive to talk with public health officials, and initiate public health responses which address the epidemic at the root cause. This is incentive-compatible as the HMO makes more money when fewer people get sick.

In the UK, the government runs such a system, and it is called the National Health Service (NHS). Under this, there is one general practitioner (GP) tied to each person. [9] On average, there are 0.58 GPs per 1000 persons. The GP is a civil servant who is paid a salary, and charges nothing to the patient. The GP's incentive is to keep the person healthy, so that the person comes back to her fewer times per year.

This is a logical and simple design that generates the correct incentives for the GP. However, the sound operation of the UK NHS requires UK-style state capacity. The UK government is able to make sure that GPs show up to work, and are conscientious about their work even though this is a transaction-intensive, discretion-intensive service. Under present Indian conditions of state capacity, this is infeasible, outside of islands of state capacity such as AIIMS.

Addressing these difficulties of healthcare requires system thinking. It is hard to see how the self-organizing system of the market economy will find